

ΠΑΙΔΙΑΤΡΙΚΕΣ ΔΟΣΕΙΣ ΦΑΡΜΑΚΩΝ

“Pediatric Drug Reference - Dosages, Side Effects & Drug Interactions”

Jane L. Gennrich, Paul D. Chan.

www.ccsublishing.com/ccs/peddrug.htm. Current Clinical Strategies Publishing.

ΦΑΡΜΑΚΕΥΤΙΚΗ ΟΥΣΙΑ	ΔΟΣΟΛΟΓΙΑ [Οδοί χορήγησης - Μέγιστη δόση - Παρενέργειες - Τοξικότητα]
Acetaminophen (Paracetamol)	10-20 mg/kg/dose PO/PR q4-6h prn. - Max: 80 mg/kg/day or 4 g/day (whichever is smaller).
Acetylcysteine (NAC)	- Acetaminophen Overdose: 140 mg/kg PO/NG x 1 dose, followed by 70 mg/kg PO/NG q4h x 17 doses. - Repeat dose if emesis occurs within 1 hr of administration.
Acyclovir	- HZV (<i>immunocompromised</i>): 500 mg/m²/dose PO 4-5 times daily or 1500 mg/m²/day IV q8h. - HZV (<i>immunocompetent</i>): 80 mg/kg/day PO 5 times daily (max: 4000 mg/day) or 30 mg/kg/day IV q8h. - VZV: 80 mg/kg/day PO qid x 5 days (max: 3200 mg/day). - Mucocutaneous HSV: 750 mg/m²/day IV q8h or 15 mg/kg/day IV q8h x 7 days. - Genital herpes or HSV (<i>immunocompromised</i>): 80 mg/kg/day PO 3-5 times daily (max: 1000 mg/day). - Herpes encephalitis: age 0-12 years: 60 mg/kg/day IV q8h. age > 12 years: 30 mg/kg/day IV q8h. <i>Infuse each IV dose over 1 hour. Adjust in renal impairment. Patients should be well hydrated to avoid urolithiasis.</i> - Topical: Apply to affected area q3h while awake for 7 days.
Adrenaline	Epinephrine (Epi-pen/Anapen, Adrenalin). - EpiPen: delivers 0.3 mg IM. EpiPen-Jr: delivers 0.15 mg IM. - Injection IM: 1 mg/mL [amp. 1:1000]. - Racemic sol. Neb: 2.25% [15-30 mL]. Sol. for inhalation: 1:1000 [30 mL]. Nebulized: 0.25-0.5 mL of 2.25% racemic epinephrine diluted in 3 mL N/S. - Continuous IV infusion: 0.1-1.0 mcg/kg/min. - CPR: 0.01 mg/kg IV/IO (max 1 mg) or high dose: 0.1 mg/kg IV/IO, 0.1 mg/kg ET. - Subcutaneous: 0.01 mg/kg using 1:1000 conc (max 0.5 mg). - EpiPen: Body Weight (BW) < 30 kg: max 0.15 mg SC (EpiPen-Jr). Body Weight (BW) > 30 kg: max 0.3 mg SC (EpiPen).
Albendazole	- Children & adults: 400 mg PO x 1. <i>For long-term therapy, monitor CBC & Liver Function Tests (LFTs) q2 weeks.</i>
Albumin	- Hypoproteinemia: 0.5-1 g/kg/dose IV infused over 2-4h, may repeat q1-2 days (25% concentration preferred, consider giving furosemide afterwards). - Hypovolemia: 0.5-1 g/kg/dose IV infused over 10-60 minutes (5% concentration preferred, dose equals 10-20 mL/kg).
Amikacin	- Age < 5 yrs (except neonates): 22 mg/kg/day IV/IM q8h. - Age 5-10 yrs: 18 mg/kg/day IV/IM q8h. - Age > 10 yrs: 15 mg/kg/day IV/IM q8h. Infuse over 30 minutes. - Therapeutic peak levels 20-30 mcg/mL & trough levels < 8 mcg/mL. <i>Adjust dose in renal impairment. Side effects include nephrotoxicity & ototoxicity.</i>

Aminophylline	• IV Loading dose: 5-6 mg/kg IV over 20-30 minutes. • IV infusion: age 1-6 months: 0.5 mg/kg/hr. ~ Age 6-12 mos: 0.6-0.75 mg/kg/hr. ~ Age 12 mos - 10 yrs: 1.0 mg/kg/hr. ~ Age 10 - 16 yrs: 0.75-0.9 mg/kg/hr. ~ Age > 16 yrs: 0.7 mg/kg/hr. • Oral dose: 1-9 yrs: 16-19.2 mg/kg/day q6-12h. ~ Age 9-12 yrs: 12.8 mg/kg/day q6-12h. ~ Age 12-16 yrs: 10.4 mg/kg/day q6-12h. ~ Age > 16 yrs: 8 mg/kg/day q6-12h. Therapeutic Levels: 10-20 mcg/mL.
Amoxicillin	• Dose: 25-50 mg/kg/day PO bid-tid (max: 500 mg/dose). • Doses as high as 80-90 mg/kg/day PO bid have been used to treat highly resistant strains of Streptococcus pneumoniae. • Anthrax infection: 80 mg/kg/day PO q8h to complete 60-day course, (max: 1500 mg/day).
Amoxi - Clav	• Otitis media: 45 mg/kg/day PO bid. • Mild-to-moderate infections: 250 mg PO tid or 500 mg PO bid. • Severe infections: 625 mg PO tid or 1000 mg PO bid. • Treating resistant Strep. pneumoniae: 80-90 mg/kg/day PO bid.
Ampicillin	• Sepsis: 100-200 mg/kg/day IV/IM q4-6h (max: 12 g/day). • Meningitis: 200-400 mg/kg/day IV/IM q4-6h (max: 12 g/day). • Oral: 50-100 mg/kg/day PO qid (max: 2-3 g/day).
Aspirin	• Kawasaki's Disease: 80-100 mg/kg/day PO/PR q6h, once fever resolves, decrease dose to 3-5 mg/kg/day PO qd. • Analgesic - Antipyretic: 10-15 mg/kg/dose PO/PR q4-6h prn (max: 4 g/day). • Anti-inflammatory: 60-90 mg/kg/day PO/PR q6h (max: 4 g/day). <i>Contraindicated in children with fever because of risk of Reye's Syndrome.</i>
Atropine	• Bradycardia (IV/ET): 0.02 mg/kg (<i>min: 0.1 mg, max: 0.5 mg in children & 1 mg in adolescents</i>), for ET administration, dilute with N/S to total volume of 2 mL. • Organophosphate: Age < 12 yrs: 0.02 - 0.05 mg/kg IV every 10-15 min, until Atropine effect. Age > 12 yrs: 1-2 mg IV initially. Double the dose every 10 min. • Ophthalmic: Instill 1-2 drops of Atropine solution prior to procedure.
Azithromycin	• Otitis Media: 5-day course, age > 6 mos: 10 mg/kg (max: 500 mg) PO on 1st day followed by 5 mg/kg (max: 250 mg) on days 2-5. • Otitis Media: 3-day course, age > 6 mos: 10 mg/kg PO qd x 3 days, (max: 500 mg/day). • Otitis Media: Single-Dose Regimen, age > 6 mos: 30 mg/kg PO x 1. • Pharyngitis Tonsillitis: age > 2 years: 12 mg/kg/day PO qd x 5 days (max: 500 mg/day). • Chlamydial Urethritis or Cervicitis, age > 12 yrs: 10 mg/kg (max: 1 g) PO x 1.
Bentonite	• Oral: 250-500 ml conc. 7%, with 60 ml Milk of Magnesia q2h.
Bisacodyl	• Age 2-11 yrs: 5-10 mg PR qd prn. • Age > 12 yrs: 10 mg PR qd prn. • Oral: age 3-11 yrs: 5 mg PO qd prn. age > 12 yrs: 5-15 mg PO qd prn. <i>Do not crush or chew tabs. Contraindicated age < 2 yrs old.</i>

Brompheniramine	• Age < 6 yrs: 0.125 mg/kg/dose q6h prn (max: 8 mg/day). • Age 6-12 yrs: 2-4 mg q6-8h prn (max: 16 mg/day). • Age > 12 yrs: 4-8 mg q4-6h prn or 12 mg SR bid (max: 24 mg/day).
Calcium chloride	• Hypocalcemia: 10-20 mg/kg/dose IV. • Hypocalcemia Secondary to Citrated Blood Transfusion: Give 0.45 mEq elemental calcium for each 100 mL of citrated blood. 10% injection contains 1.36 mEq calcium/mL. Maximum infusion rate 80 mg/kg/hr. May not be given IM or SC.
Calcium gluconate	• Hypocalcemia: 200-500 mg/kg/day IV as a continuous infusion or q6h. • 500-725 mg/kg/day PO tid-qid.
Captopril	• Neonates: 0.05-0.1 mg/kg/day PO q8-24h. • Infants: Initially 0.15-0.3 mg/kg/dose, max: 6 mg/kg/day q6-24h. • Children: Initially 0.3-0.5 mg/kg/dose (max: 12.5-25 mg), titrate dose upward to max of 6 mg/kg/day q6-24h. • Adolescents: Initially 12.5-25 mg q8-12h, titrate dose upward by 25 mg/dose to max of 450 mg/day.
Carbamazepine	• Age < 6 years: Initially 10-20 mg/kg/day PO q6-12h, dosage may be increased every 7 days up to 35 mg/kg/day. • 6-12 yrs: Initially 100 mg PO bid or 10 mg/kg/day PO bid or 50 mg suspension PO qd. May increase by 100 mg/day at weekly intervals until optimal response and therapeutic levels are achieved. Usual range 15-30 mg/kg/day PO q6-12h (max: 1000 mg/day). • Age > 12 years: Initially 200 mg PO bid, increase by 200 mg/day at weekly intervals until therapeutic levels achieved, usual dose 800-1200 mg/day q6-8h (max: 2400 mg/day). Therapeutic Range: 4-12 mcg/mL.
Cefaclor	• 20-40 mg/kg/day PO q8-12 (max: 2 gm/day). • Adolescents: 250-500 mg PO q8h or 375 mg ER tab PO q12h.
Cefepime	• 100 mg/kg/day IV/IM q12h (max: 4 g/day). • 150 mg/kg/day q8h (max: 6 g/day) in cystic fibrosis patients.
Cefotaxime	• 100-200 mg/kg/day IV/IM q6-8h (max: 12 g/day). • Meningitis: 200 mg/kg/day IV q6h.
Cefoxitin	• Mild-moderate infection: 80-100 mg/kg/day IV/IM q6-8h (max: 12 g/day) • Severe infection: 100-160 mg/kg/day IV/IM q4-6h (max: 12 g/day).
Cefpodoxime	• Age 6 mos-12 yrs: 10 mg/kg/day PO bid (max: 800 mg/day). • Age > 12 yrs: 100-400 mg/day PO bid. • Uncomplicated gonorrhea: 200 mg PO x 1.
Cefprozil	• 30 mg/kg/day PO bid (max: 1000 mg/day).
Ceftizoxime	• 150-200 mg/kg/day IV/IM q6-8h (max: 12 g/day).
Ceftriaxone	• 50-100 mg/kg/day IV/IM q12-24h (max: 4 g/day). • Meningitis: 100 mg/kg/day.
Cefuroxime	• Otitis media: Susp: 30 mg/kg/day PO bid (max: 1 g/day) Tab: 250 mg PO bid. • Pharyngitis, tonsillitis: Suspension: 20 mg/kg/day PO bid (max: 500 mg/day) Tab: age: 3 mos-12 yrs: 125 mg PO bid, age > 12 yrs: 250-500 mg PO bid. Crushed tablets are very bitter and not very palatable.

Cetirizine	• Age 2-5 yrs: 2.5 mg (5 drops Zirtek®) PO qd or bid. • Age > 6 yrs: 5-10 mg (10-20 drops Zirtek®) PO qd or bid.
Charcoal activated	• 1 g/kg/dose (max: 50 g) PO/NG. <i>First dose should be given using product containing sorbitol as a cathartic. Repeat doses may be advised for enterohepatically excreted agents.</i>
Cimetidine	• 20-40 mg/kg/day PO/IV/IM q6h (max: 2400 mg/day) • Adolescents - Ulcer Treatment: 300 mg IV q6h, or 300 mg PO qid, or 400 mg PO bid. Ulcer prophylaxis: 400-800 mg PO qhs. • GERD: 400 mg PO q6h or 800 mg PO q12h.
Ciprofloxacin	• PO: 20-30 mg/kg/day bid, in Cystic Fibrosis may use 40 mg/kg/day bid [max: 1500 mg/day] • IV: 20-30 mg/kg/day bid, [max: 800 mg/day], in Cystic Fibrosis: 30 mg/kg/day, [max: 1200 mg/day]. • Anthrax Treatment: 20-30 mg/kg/day IV q12h x 60 days, max: 1000 mg/day. Substitute PO for IV (same dose) as soon as clinical condition improves. • Ophthalmology: Instill 1-2 drops in affected eye(s) q2h while awake for 2 days, then 1-2 drops q4h while awake for 5 more days OR apply oint. tid x 2 days, then bid x 5 days. <i>Not recommended for use in patients < 18 years.</i>
Clarithromycin	• Dose: 15 mg/kg/day PO bid (max: 1 g/day).
Clindamycin	• PO: 10-30 mg/kg/day q6-8h (max: 1800 mg/day). • IV/IM: 25-40 mg/kg/day q6-8h (max: 4800 mg/day).
Desloratadine	• Age > 12 yrs: 5 mg PO, qd. <i>Rapidly disintegrating tablet dissolves on the tongue without water.</i> • Children 1 - 5 yrs: 2.5 ml (1.25 mg) oral solution, qd. • Children 6 - 11 yrs: 5 ml (2.5 mg) oral solution, qd. <i>The safety & efficacy of Desloratadine (Aerius®) 0.5 mg/ml oral solution in children below the age of 1 year have not been established.</i>
Dexamethasone	• Airway edema: 0.5-2 mg/kg/day IV/IM/PO q6h. • Antiemetic (chemotherapy): 10 mg/m² IV x 1, then 5 mg/m² IV q6h prn. • Anti-inflammatory: 0.08-0.3 mg/kg/day PO/IV/IM or 2.5-10 g/m²/day PO/IV/IM q6-12h. • Bacterial meningitis: 0.6 mg/kg/day IV q6h x 4 days. • Cerebral edema: 1-2 mg/kg/dose IV x 1, then 1-1.5 mg/kg/day IV q4-6h (max: 16 mg/day). • Physiologic replacement: 0.03-0.15 mg/kg/day PO/IV/IM or 0.6-0.75 mg/m²/day PO/IV/IM q6-12h.
Diazepam	• Anxiety or Sedation: 0.12-0.8 mg/kg/day PO q6-8h or 0.04-0.3 mg/kg/dose IV q4h prn • Status epilepticus (IV): 0.05-0.3 mg/kg/dose q15-30 min to max cumulative dose of 5 mg (age < 5 yrs) or 10 mg (age > 5 yrs). • Status epilepticus (rectal gel): age 2-5 yrs: 0.5 mg/kg/dose (max: 10 mg) age 6-11 yrs: 0.3 mg/kg/dose (max: 10 mg) age > 12 yrs: 0.2 mg/kg/dose (max: 20 mg). • Conscious sedation: 0.2-0.3 mg/kg PO/IV/IM (max: 10 mg) 45-60 min prior to procedure. <i>May cause significant respiratory depression.</i>
Dicloxacillin	• Dose: 25-100 mg/kg/day PO qid (max: 2000 mg/day). <i>Suspension is unpalatable. Open capsule & sprinkle powder on food for young children if possible. Take on an empty stomach.</i>

Dimenhydrinate	Age > 12 yrs: 50-100 mg PO q4-6h prn (max: 400 mg/day Vomex-A®).
Dimercaprol (BAL)	- Lead Poisoning (use with edetate calcium disodium-EDTA): ~ Mild: 3 mg/kg/dose IM q4h x 2-7days. ~ Severe: 4 mg/kg/dose IM q4h x 2-7days. - Arsenic or Gold Poisoning: ~ Mild: mg/kg/dose IM q6h x 2 days, then q12h x 1 day, then qd x 10 days. ~ Severe: 3 mg/kg/dose IM q4h x 2 days, then q6h x 1 day, then q12h x 10 days - Mercury Poisoning: 5 mg/kg/dose IM x 1 (1st day), then 2.5 mg/kg/dose IM q12-24h x 10 days. <i>Give undiluted by deep IM injection. May not give IV.</i>
Diphenhydramine	1 mg/kg/dose IM/IV/PO qid (max: 50 mg/dose Benadryl®).
Doxycycline	- Age > 8 yrs: 2-4 mg/kg/day IV/PO bid (max: 200 mg/day). - Anthrax Post-exposure Prophylaxis: < 45 kg: 4.4 mg/kg/day PO bid x 60 days > 8 yrs & > 45 kg: 100 mg PO bid x 60 days. <i>Use of Tetracyclines (Vibramycin®) in childhood (patients < 8 years of age) may cause permanent dental discoloration and enamel hypoplasia.</i>
EDTA	- Lead Poisoning Encephalopathy (Calcium EDTA): 250 mg/m²/dose IM q4h or 50 mg/kg/day continuous IV infusion or 1-15 gm/m² IV as either an 8 hr or 24 hr infusion. <i>Concomitant treatment with BAL is necessary.</i> - Hypercalcemia (EDTA disodium): 40-70 mg/kg (max: 3 g) IV over 3-4 hours. May repeat daily for five days.
Famciclovir	- Herpes zoster-HZV (adolescents): 500 mg PO tid for 7 days. - Genital herpes: 250 mg PO tid x 7-10 days - Genital herpes, recurrent episodes (adolescents): 125 mg PO bid for 5 days. - Daily suppressive therapy: 250-500 mg/day PO bid x 1 year, then reassess for recurrence.
Fentanyl	Dose: 1-2 mcg/kg/dose (max: 100 mcg/dose) IV q1-2h prn or 1-2 mcg/kg/hr continuous IV infusion (may need to titrate to higher dose). <i>Reversible with Naloxone (Narcan®).</i>
Ferrous gluconate	- Children (iron deficiency anemia): ~ Mild-Moderate: 3 mg/kg/day PO qd-bid. ~ Severe: 4-6 mg/kg/day PO tid. ~ Prophylaxis: 1-2 mg/kg/day PO qd. <i>Contains 12% elemental iron.</i>
Ferrous sulfate	- Oral dosages in mg elemental iron: Premature infants: 7.5-15 mg/day qd-bid. <i>Doses at the higher end of the therapeutic range may be necessary for infants on concomitant erythropoietin. Contains 20% elemental iron.</i> - Children (Iron deficiency anemia): ~ Mild-moderate: 3 mg/kg/day PO qd-bid. ~ Severe: 4-6 mg/kg/day PO tid. ~ Prophylaxis: 1-2 mg/kg/day PO qd.
Fludrocortisone	Infants & Children: 0.05-0.1 mg/day PO qd.
Flumazenil	- Reversal of Benzodiazepine: Children: 0.01 mg/kg IV repeat as needed, (max: 0.2 mg). - Adolescents: 0.2 mg IV over 30 seconds, wait 30 seconds, then give 0.3-0.5 mg over 30 seconds if needed. Continuous infusion: 0.005-0.01 mg/kg/hr. <i>May need to repeat doses because the half-life of Flumazenil (Anexate®) may be significantly shorter than the half-life of the benzodiazepine being reversed.</i>

Fluticasone*	- MDI: 1-2 puffs bid. - Nasal: age > 4 yrs: 1 spray in each nostril qd, may increase to 2 sprays in each nostril qd. Once symptoms are controlled, reduce dose to 1 spray in each nostril qd. Nasal pump must be primed before first use or after > one week of non-use by actuating six times. - Diskus: age 4-11 yrs: 50 mcg bid, may increase to 100 mcg bid. - Diskus: age > 12 yrs: 100 mcg bid, may increase to max 500 mcg bid. <i>* Dry powder inhaler - cannot be used with spacer.</i> <i>* Not indicated for relief of acute bronchospasm.</i>
Folic acid	- Folic Acid Deficiency (PO/IV/IM/SC): ~ Infants: 15 mcg/kg/dose or 50 mcg qd. ~ Children 1-10 yrs: Initial: 1 mg/day qd, maint: 0.1-0.4 mg/day qd. ~ Age > 11 yrs: Initial: 1 mg/day qd, maint: 0.5 mg/day qd.
Fomepizole	15 mg/kg IV loading dose, then 10 mg/kg IV q12h x 4 doses, then 15 mg/kg IV q12h. <i>Methanol or ethylene glycol ingestion does not require hemodialysis.</i>
Formoterol	Age > 5 yrs: 12 mcg inhaled bid.
Gabapentin	- Anticonvulsant: ~ 3-12 yrs: 10-15 mg/kg/day PO tid, titrate as tolerated q3d (max 50 mg/kg/day) ~ Age > 12 yrs: 300 mg PO tid, titrate if necessary. - Usual dose 800-1800 mg PO tid, max: 3600 mg/day - Neuropathic Pain: 5 mg/kg PO qhs (1st day), 5 mg/kg/dose PO bid (2nd day), 5 mg/kg/dose PO tid (3rd day). Titrate to effect. - Usual range 8-35 mg/kg/day PO tid; max: 1800 mg/day.
Ganciclovir	- CMV Retinitis: Induction: 10 mg/kg/day IV bid x 14-21 days. Maintenance: 5 mg/kg/day IV q24h x 7 days/week or 6 mg/kg/day IV q24h x 5 days/week. - Oral maintenance following IV induction: 6 mos-16 yrs: 30 mg/kg/dose PO q8h, Age > 16 yrs: 1 g tid or 500 mg q3h while awake (6 times daily). - Other CMV Infections: ~ Induction: 10 mg/kg/day IV q12h or 7.5 mg/kg/day IV q8h x 14-21 days. ~ Oral maintenance following IV induction: 6 mos-16 yrs: 30 mg/kg/dose PO q8h, age >16 yrs: 1000 mg tid or 500 mg q3h while awake (6 times daily)
Gentamicin	- Age < 5 yrs (except neonates): 7.5 mg/kg/day q8h, IV/IM. - Age 5-10 yrs: 6.0 mg/kg/day q8h, IV/IM. - Age > 10 yrs: 5.0 mg/kg/day q8h, IV/IM. - Therapeutic peak levels 4-10 mcg/mL; trough levels < 2 mcg/mL. <i>Adjust dose in renal failure, potential side effects include nephrotoxicity & ototoxicity.</i> - Ophthalmology: Apply ointment to conjunctival sac bid-tid or instill 1-2 drops into eye(s) q4h. - Topical: Apply oint. topically, bid-qid.
Glucagon	- Hypoglycemia: < 20 kg: 0.02-0.03 mg/kg/dose (max: 0.5 mg) IM/IV/SC. May repeat in 20 minutes if needed. - BW > 20 kg: 1 mg IM/IV/SC. May repeat in 20 minutes if needed.
HES 6%	10 mL/kg/dose IV over at least 30 minutes. - Maximum daily dose 20 mL/kg. Diluted in normal saline (Na⁺ 154 mEq/lit).
Hydralazine	IV/IM: Initially 0.1-0.2 mg/kg/dose (max: 20 mg) q4-6h prn, may increase to 1.7-3.5 mg/kg/day q4-6h prn (max: 40 mg/dose).

Hydrocortisone	• Status asthmaticus: 1-2 mg/kg/dose IV/IM q6h x 24 hours, then decrease dose to 0.5 mg/kg/dose IV/IM q6h. • Anti-inflammatory: 1-5 mg/kg/day IV/IM q12-24h. • Acute adrenal insufficiency: 1-2 mg/kg IV/IM x 1, then 25-150 mg/day IV/IM q6-8h.
Hydroxyzine	• 2 mg/kg/day PO q6-8h (max: 100 mg/dose Atarax®). • 0.5-1 mg/kg/dose IV/IM q4-6h (max: 100 mg/dose Atarax®).
Ibuprofen	• Antipyretic & Analgesic: 5-10 mg/kg/dose PO q6-8h (max 600 mg/dose) <i>Administer with food to decrease GI upset.</i> <i>May alternate with acetaminophen to control fever.</i> • Juvenile Rheumatoid Arthritis: 30-50 mg/kg/day PO q6h (max 2.4 gm/day).
Immune globulin IVIG	• Kawasaki: 2 g/kg IV as a single dose (<i>preferred</i>) or 400 mg/kg/day x 4 days. • Immunodeficiency syndrome: 200-400 mg/kg/dose IV q2-4 wk. • Idiopathic Thrombocytopenic Purpura (ITP): 400-1000 mg/kg/day x 2-5 days, maintenance dose: 400-1000 mg/kg/dose q3-6 wk based on clinical response & platelet count. • Severe viral & bacterial infections: 500-1000 mg/kg x 1, repeat in 1 week prn.
Ipratropium	• Nebulizer: Age 3-12 yrs: 125-500 mcg tid Age > 12 yrs: 500 mcg tid-qid. • MDI: age 3-12 yrs: 1-2 puffs tid age > 12 yrs: 2 puffs qid up to 12 puffs/day.
Lactulose	• Infants: 2.5-10 mL/day PO divided 3-4 times/day. Adjust dosage to produce 2-3 soft stools per day. • Children: 40-90 mL/day PO divided 3-4 times/day. Adjust dosage to produce 2-3 soft stools per day.
Lansoprazole	• GERD: BW < 10 kg: 7.5 mg PO qd BW 10-20 kg: 15 mg PO qd BW > 20 kg: 30 mg PO qd Age > 12 yrs: 15-30 mg PO qd. • H. pylori: BW > 20 kg: 30 mg PO bid (<i>with Amoxicillin & Clarithromycin</i>).
Levetiracetam	• Age 4-16 yrs: 10-20 mg/kg/day PO bid (max: 1000 mg/day), may increase by 10-20 mg/kg/day q2 weeks to max of 60 mg/kg/day bid, (max: 3000 mg/day). • Age > 16 yrs: 500 mg PO bid, may increase as needed to max of 3000 mg/day.
Linezolid	• Complicated skin infections, pneumonia, VRE infections: ~ Birth-11 yrs: 30 mg/kg/day PO/IV q8h ~ Age > 12 yrs: 600 mg PO/IV q12h. • Uncomplicated skin infection: ~ Age < 5 yrs: 30 mg/kg/day PO/IV q8h ~ Age 5-11 yrs: 20 mg/kg/day PO/IV q12h ~ Age 12-17 yrs: 600 mg PO q12h
Loperamide	• Acute Diarrhea: age 2-5 yrs: 1 mg PO tid age 6-8 yrs: 2 mg bid age 8-12 yrs: 2 mg tid age > 12 yrs: 4 mg x 1 and 2 mg after each loose stool (max: 16 mg/day). Discontinue if no improvement in 48 hrs. • Chronic diarrhea: 0.08-0.24 mg/kg/day PO bid-tid; max: 2 mg/dose.

Loratadine	• Age 2-5 yrs (BW: 30 kg or less): 5 mg (or 5 ml of the syrup 5 mg / 5 ml) PO qd. • Age > 6 yrs: 10 mg (or 10 ml of the syrup 5 mg / 5 ml) PO qd.
Magnesium sulfate (MgSO₄)	• 100-200 mg/kg/dose PO q6h prn. • 25-50 mg/kg/dose IV q4-6h prn (max single dose: 2 g MgSO₄). • Daily Maintenance (IV): 30-60 mg/kg/day in maintenance IV solution. • Maximum infusion rate 125 mg/kg/hr.
Mannitol	• 0.25-1 gm/kg/dose IV q4-6h prn. <i>Use an in-line filter when infusing.</i>
Mebendazole	• Age > 2 yrs - Pinworms: 100 mg PO (single dose), may repeat dose in 2 weeks. • Age > 2 yrs - Other helminths: 100 mg PO bid x 3 days. Repeat in 3-4 weeks.
Methylprednisolone	• [Medrol] - Anti-inflammatory: 0.5-1.7 mg/kg/day PO q6-12h or 5-25 mg/m²/day PO q6-12h (usual max 60 mg/day). • [Lyo-Drol] - Status Asthmaticus: 2 mg/kg/dose IV/IM q6h x 1-4 doses, then 0.5-1 mg/kg/dose IV/IM q6h (usual max: 125 mg/dose). • Anti-inflammatory: 0.5-1.7 mg/kg/day IV/IM q6-12h. • SLE Nephritis: 30 mg/kg IV qod x 6 doses. Each dose over at least 30 min. • Acute spinal cord injury: 30 mg/kg IV over 15 min, followed by 5.4 mg/kg/hr infusion for 23 hrs.
Metoclopramide	• Gastroesophageal Reflux: 0.4-0.8 mg/kg/day qid (PO/IM/IV) (max: 10 mg/dose). • Antiemetic, chemotherapy induced: 1 mg/kg IV q4h prn. <i>To prevent extrapyramidal reactions associated with this high dose, pretreat with Diphenhydramine (Benadryl®) 1 mg/kg IV.</i>
Metronidazole	• Anaerobic infections: 30 mg/kg/day PO/ IV q6-8h (max: 4 g/day). • Cl. difficile: 20-35 mg/kg/day PO/IV q6h (max: 4 g/day). PO is preferred. • Amebiasis: 35-50 mg/kg/day PO tid (max: 2250 mg/day) • Other parasitic infections: 15-30 mg/kg/day PO tid (max: 750 mg/day). • H. pylori: 15-20 mg/kg/day PO bid x 4 weeks, (max: 1500 mg/day), with Amoxicillin & Bismuth subsalicylate.
Midazolam	• Preoperative Sedation: ~ IV: 0.05-0.1 mg/kg/dose, repeat q2-3 min prn, to total dose 0.1-0.2 mg/kg. ~ IM: 0.05-0.15 mg/kg 30-60 min before surgery (max: 5 mg/dose) • Sedation: 0.1 mg/kg IV q1-2h prn or loading dose 0.05-0.2 mg/kg IV, then continuous infusion 0.05-0.12 mg/kg/hr. Titrate as needed. • Oral: 0.2-0.4 mg/kg (max: 15 mg) 30-45 min prior to procedure. • Intranasal: 0.2 mg/kg x 1 (5 mg/mL injectable product in a needleless syringe).
Montelukast	• Age 12-23 mos: 4 mg PO qhs. • Age 2-5 yrs: 4 mg PO qhs. • Age 6-14 yrs: 5 mg PO qhs. • Age >14 yrs: 10 mg PO qhs.
Naloxone	• Opioid Intoxication: 0.1 mg/kg IV/IM/ET/SC (max: 2 mg). Repeat dose in 2-5 min as needed. <i>If no improvement after 3 doses (Narcan®), a non-opioid drug overdose should be suspected.</i> • Post-anesthesia narcotic reversal: 0.01 mg/kg IV, may repeat q2-3 minutes.
Nifuroxazide	• Age 1 - 11 mos: 2.5 ml of oral susp. tid x 5-7d, PO. • Age 1 - 10 yrs: 5 ml of oral susp. tid x 5-7d, PO. • Age > 10yrs: 5 ml of oral susp. q6-8h or 1 caps. 200 mg q6-8h, for a week.

Nitrofurantoin	- Over 1 mos: Acute infection: 5-7 mg/kg/day PO qid (max: 400 mg/day). - Chronic suppressive therapy for UTIs: 1-2 mg/kg/day PO q24h, (max: 100 mg/day).
Norepinephrine	0.05-2 mcg/kg/min continuous IV infusion, titrate to desired effect.
Nystatin	- Oral candidiasis: ~ Infants: 1 mL of susp in each side of the mouth qid. ~ Children: 5 mL swish and swallow qid. ~ Adolescents: 5-10 mL swish and swallow qid.
Omeprazole	1 mg/kg/day PO q12-24h (max: 20 mg/day for duodenal ulcer, 40 mg/day for gastric ulcer, 60 mg/day for hypersecretory conditions). - Helicobacter pylori: 15-30 kg: 10 mg PO bid, > 30 kg: 20 mg PO bid.
Ondansetron	- Chemotherapy Induced Nausea (IV): 0.15 mg/kg/dose, given 30 min prior to chemotherapy & 4 hr & 8 hr later or 0.3 mg/kg/dose x 1 thirty min prior to chemotherapy or for severe cases, 0.15 mg/kg x 1, followed by 0.45 mg/kg/day (max: 32 mg/day) as a continuous IV infusion. - Post-operation nausea-vomiting (IV): >2 yrs & <40 kg: 0.1 mg/kg IV x 1, >40 kg: 4 mg IV x 1. - Oral: 4-11 yrs: 4 mg/dose PO 30 min prior to chemotherapy, may repeat 4h & 8h after 1st dose. Age > 11 years: 8 mg/dose PO 30 min prior to chemotherapy, may repeat 4h & 8h after 1st dose.
Penicillamine	- Lead Poisoning: 20-30 mg/kg/day PO q6-8h (max: 1.5 gm/day). - Rheumatoid Arthritis: Initially 3 mg/kg/day (max: 250 mg/day) PO bid for 3 months, then 6 mg/kg/day PO (max: 500 mg/day) bid for 3 months, then may increase to the maximum of 10 mg/kg/day PO tid-qid (max: 1500 mg/day).
Penicillin G	- Group A Streptococcal pharyngitis: 25,000-50,000 IU/kg IM, as a single dose (max: 1.2 million units). - Systemic Infections: ~ IV/IM: 100,000-250,000 U/kg/day q4-6h, for severe infections may use up to 400,000 U/kg/day (max: 24 million U/day). ~ 25-50 mg/kg/day PO q6-8h (max: 3 g/day). - Prophylaxis of Pneumococcal Infections: ~ Age <2 mos to 3 yrs: 125 mg PO bid. ~ Age 4-5 yrs: 250 mg PO bid. ~ Age >5 yrs: 250 mg PO bid.
Phenobarbital	- Status Epilepticus: 15-18 mg/kg IV in a single or divided dose, may give additional 5 mg/kg/dose every 15-30 min, until seizure is controlled or a total dose of 30 mg/kg is reached. Respiratory support may be needed. - Anticonvulsant Maintenance: ~ Infants: 5-6 mg/kg/day PO/IV q12-24h. ~ Age 1-5 yrs: 6-8 mg/kg/day PO/IV q12h. ~ Age 5-12 yrs: 5 mg/kg/day PO/IV qd-bid. ~ Age >12 yrs: 1-3 mg/kg/day PO/IV qd-bid. - Sedation: 2 mg/kg/dose PO tid (max 120 mg/day). - Hypnotic (IM/IV/SC): 3-5 mg/kg qhs. Adults: 100-320 mg qhs. - Hyperbilirubinemia: <12 yrs: 3-8 mg/kg/day PO q12h. <i>Therapeutic serum levels: 15-40 mcg/mL.</i>

Phenytoin	• Status Epilepticus: 15-18 mg/kg IV in a single or divided dose. Oral Loading dose: 15-20 mg/kg PO, administer oral loading dose in 3 divided doses given q2-4h. • Maintenance, Infants & Children: Initially 5 mg/kg/day PO/IV q12h, may need to increase up to 10 mg/kg/day PO/IV q8-12h. ~ <i>Therapeutic serum levels: 10-20 mcg/mL.</i> ~ <i>Maximum infusion rate: 1 mg/kg/min (up to max rate 50 mg/min).</i>
Prednisolone	• Acute asthma: 1-2 mg/kg/day PO qd-bid or 2-4 mg/kg/day IV q6-8h. • Anti-inflammatory or immunosuppressive: 0.1-2 mg/kg/day PO/IV qd-qid.
Propofol	• IV induction: 2.5-3.5 mg/kg over 30 seconds. • Maintenance IV infusion: 125-300 mcg/kg/min.
Ranitidine	• IV: 2-4 mg/kg/day q6-8h (max: 200 mg/day), may put daily dose in TPN. • Continuous IV infusion: loading dose 1 mg/kg, followed 0.08-0.17 mg/kg/hr. • PO: 4-5 mg/kg/day q8-12h (max: 10 mg/kg/day) or PO: 300 mg/day for GERD or 600 mg/day for erosive esophagitis.
Rifaximin	• Age < 12 years: Safety and efficacy not established. • Age ≥ 12 years: 200 mg PO q8hr for 3 days, with or without food.
Salbutamol	• Inhalation: 1-2 puffs q4-6h prn; spacers recommended for young children. • Oral: age 2-6 yrs: 0.1-0.2 mg/kg/dose tid (max 4 mg/day). age 6-12 yrs: 2 mg/dose tid-qid or 4 mg ER bid. age > 12 yrs: 2-4 mg/dose tid-qid or 4-8 mg ER bid. • Nebulizer: 0.01-0.05 cc/kg of 0.5% sol (min: 0.1 cc, max: 1 cc) q4-6h prn.
Salmeterol	• MDI: > 4 yrs: 2 puffs bid. • Diskus: > 4 yrs: 1 puff bid. <i>Can not use spacer with Diskus for inhalation.</i>
Sodium bicarbonate (NaHCO₃⁻)	• Cardiac Arrest: 1 mEq/kg IV initially, may repeat 0.5 mEq/kg IV after 10 min (max: 50 mEq/dose). <i>Caution: patients must be adequately ventilated before administering sodium bicarbonate.</i> • Chronic Renal Failure: 1-3 mEq/kg/day PO bid-qid. • Urinary Alkalinization: 120-200 mEq/m²/day diluted in maintenance IV fluids of 3000 mL/m²/day to maintain urine pH in the desired range.
Sodium chloride	• Maintenance Na⁺ Requirements (PO/IV): 3-4 mEq/kg/day (max: 100-150 mEq/day). • Acute Hyponatremia: Maximum infusion rate: 1 mEq/kg/hr.
Sodium polystyrene (Kayexalate)	• 0.5-1 g/kg PO/PR q6h prn (max: 15 g/dose PO & 50 g/dose PR). • 1 g resin binds 1 mEq potassium. <i>Suspension can be used orally or rectally.</i>
Succinylcholine	• IV: Initial dose 1-2 mg/kg (max: 150 mg), • IV: maintenance dose 0.3-0.6 mg/kg IV prn. • IM: 2.5-4 mg/kg (max: 150 mg).
Tetracycline	• PO: age > 8 yrs: 25-50 mg/kg/day q6h (max: 3 g/day).
Thiopental	• Increased Intracranial Pressure: 1.5-5 mg/kg/dose IV prn. • Seizures: 2-3 mg/kg/dose IV prn. • Sedation: 5-10 mg/kg/dose PR (max 4 g).
Ticarcillin	• 200-300 mg/kg/day IV/IM, q4-6h. Doses 400 mg/kg/day IV/IM, q4-6h prn, have been used in cystic fibrosis. Maximum daily dose 24 g. <i>Adjust dosage in renal impairment. Contains 5.2-6.5 mEq sodium per gram.</i>

Tobramycin	• IV/IM: Infuse IV dose over 30 minutes. <i>Therapeutic peak serum levels: 4-10 mcg/mL.</i> ~ Age <5 yrs: 7.5 mg/kg/day q8h. ~ Age 5-10 yrs: 6.0 mg/kg/day q8h. ~ Age >10 yrs: 5.0 mg/kg/day q8h. • Ophth: Instill 1-2 drops into eye(s) q4h, or apply ointment to conjunctival sac bid-tid. The ophthalmic solution can be used in the ears. • Nebulizer (<i>cystic fibrosis patients</i>): > 6 yrs: 300 mg nebulized bid in repeating cycles of 28 days on drug, 28 days off drug.
Topiramate	• Age 2-16 yrs: 1-3 mg/kg/day (max: 25 mg/day) PO qhs x 1 wk, then increase q1-2 wks by 1-3 mg/kg/day bid, usual maintenance dose 5-9 mg/kg/day bid. • Age >16yrs: Initially 25-50 mg PO qhs x 1 wk, then increase by 25-50 mg/day bid x 1 wk, then increase by 50 mg/day until effective dose (<i>usual maintenance 200 mg bid, max 1600 mg/day</i>). <i>Adequate fluid intake is necessary to prevent kidney stone formation.</i>
Trimethoprim	• Otitis Media: 10 mg/kg/day PO q12h (max: 200 mg/day) • UTI: Age <12 yrs: 4-6 mg/kg/day PO bid. Age >12 yrs: 100 mg PO bid or 200 mg PO qd.
Valacyclovir	• Adolescents: ~ Herpes Zoster: 1 gm PO tid for 7 days. ~ Herpes labialis (cold sores): 2 g PO bid x 1 day. ~ First episode genital herpes: 1 g PO bid x 10 days. ~ Recurrent genital herpes: 500 mg PO bid for 3 days. • Chronic Suppressive Therapy: 500-1000 mg/day PO qd.
Valproic acid	• Seizures: ~ PO: Initially 10-15 mg/kg/day qd-tid; increase dose 5-10 mg/kg/day q1wk to maintenance (usual range 30-60 mg/kg/day bid-tid, may require up to 100 mg/kg/day in combination with other anticonvulsants). ~ PR: May use oral syrup (Depakin®) rectally (<i>dilute 1:1 with water</i>): loading dose 17-20 mg/kg, maintenance dose 10-15 mg/kg/dose q8h. ~ IV: 10-60 mg/kg/day IV q6h, if already stable on oral regimen but now NPO (<i>"nothing by mouth"</i>), use same mg/kg/day as oral but divide q6h. <i>Therapeutic Serum Levels: 50-100 mcg/mL.</i> <i>Life-threatening pancreatitis may occur. Monitor liver enzymes and CBC.</i>
Vancomycin	• Gram-positive infection: 40-60 mg/kg/day IV q6-8h (max: 2 g/day). <i>Adjust dose in renal impairment.</i> • Clostridium difficile colitis: 40 mg/kg/day PO q6-8h (max: 2 g/day). <i>Oral formulation is not effective for systemic infection.</i>
Vecuronium	• IV: 0.1 mg/kg/dose prn. • Continuous IV infusion: 0.05-0.1 mg/kg/hr, titrate to maintain paralysis.

Επεξήγηση συντμήσεων των δοσολογιών

bid: <i>bis in die</i>	2 φορές την ημέρα	qhs: <i>quaque hora somni</i>	Κάθε προ του ύπνου
tid: <i>ter in die</i>	3 φορές την ημέρα	prn: <i>pro re nata</i>	Όπου απαιτείται
qh: <i>quaque hora</i>	Κάθε ώρα	pr: <i>per rectum</i>	Ενδοορθικά
qd: <i>quaque in die</i>	1 φορά την ημέρα	ng: <i>nasogastric</i>	Ρινογαστρικό
qid: <i>quarter in die</i>	4 φορές την ημέρα	io: <i>intraosseous</i>	Ενδοοστικά
qod: <i>quaque altera die</i>	Ημέρα παρ' ημέρα	et: <i>endotracheal</i>	Ενδοτραχειακά